

March 19, 2026

Chairperson Christina Henderson
Committee on Health
1350 Pennsylvania Ave, Suite 402, NW
Washington, DC 20004

Councilmember Charles Allen
1350 Pennsylvania Avenue, Suite 110, NW
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Councilmember Wendell Felder
1350 Pennsylvania Ave, Suite 406, NW
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Councilmember Brianne Nadeau
1350 Pennsylvania Avenue, Suite 108, NW
Washington, DC 20004

Councilmember Zachary Parker
1350 Pennsylvania Avenue, Suite 102, NW
Washington, DC 20004

RE: Letter in Opposition to District of Columbia B26-0525

Dear Chairperson Henderson, Councilmember Allen, Councilmember Felder, Councilmember Nadeau, and Councilmember Parker:

On behalf of the advertising industry, we write to oppose District of Columbia B26-0525.¹ We provide this letter to offer our non-exhaustive list of concerns about this bill. As drafted, B26-0525 would significantly frustrate District of Columbia (“DC”) consumers and hinder businesses’ attempts to reach them with legitimate and lawful advertising. As presently drafted, the bill would have far-reaching, unintended, and unfavorable consequences for District residents and the business community alike. Accordingly, we ask you to decline to advance the bill as drafted out of the Committee on Health (“Committee”).

As the nation’s leading advertising and marketing trade associations, we collectively represent thousands of companies across the country. These companies range from small businesses to household brands, advertising agencies, and technology providers. Our combined membership includes more than 2,000 companies that power the commercial Internet, which accounted for nearly 20 percent of total U.S. gross domestic product (“GDP”) in 2024.² By one estimate, approximately 14.1% of DC jobs in 2024 were related to the ad-subsidized Internet, a share projected to increase to 15.5% by 2029.³ Our group has more than a decade’s worth of hands-on experience it can bring to bear on matters related to consumer privacy and controls. We would welcome the opportunity to engage with the Committee further on the points we discuss in this letter.

I. B26-0525’s definition of “personal health data” is overly broad.

¹ District of Columbia B26-0525 (2026 Session), located [here](#) (hereinafter, “B26-0525”).

² S&P Global, THE ECONOMIC IMPACT OF ADVERTISING ON THE US ECONOMY, 2024-2029 at 4 (Aug. 2025), located at https://theadcoalition.com/wp-content/uploads/2025/08/TAC_SP-Global-Final-Report_August-2025.pdf.

³ *Id.* at 15-16.

B26-0525 would establish a new, overly broad definition of “personal health data,” which could unintentionally impede DC residents from receiving useful and relevant information about products and services they may desire. As defined, the term “personal health data” would include any information that could possibly be related—however tangentially—to the health of an individual.⁴ The definition could be interpreted to include basic data points, such as the fact that an individual purchased non-prescription shampoo for individuals with dry hair at a local grocer, attended a fitness class, or signed up to receive promotional notices about specific clothing or footwear restocks. The routine use of this kind of information is expected by consumers looking for convenience and savings, but B26-0525’s broad definition of “personal health data” could sweep such information into its ambit.

The Committee should consider updating the bill to narrow the definition of the term “personal health data.” We recommend that the Committee align the bill’s definition of “personal health data” with the majority of states that have passed legislation regulating consumer health data, such as the definitions of the term in the Connecticut, Maryland, and Nevada laws.⁵ In those state laws, “consumer health data” is personal information that is linked or reasonably capable of being linked to a consumer and that “*is used to identify*” the past, present, or future health status of the consumer. This definition would help ensure that the term is cabined to information actually used to identify a consumer’s health status and that less sensitive data points are not unintentionally swept into the definition of the term.

II. B26-0525 would prohibit advertising to consumers who are in or near entities providing in-person health care services or supplies.

B26-0525 would prohibit any person or entity from implementing a geofence around a location that provides in-person health care services where the geofence is used to identify or track individuals seeking such services, collect personal health data from an individual, or send notifications, messages, or advertisements to an individual related to the individual’s health data or health care services.⁶ As discussed above, the definition of “personal health data” is so broad that this geofencing restriction may unintentionally encompass non-sensitive data, and therefore impact general advertising practices that do not involve identifying or targeting individuals based on actual health conditions. Whole neighborhoods around hospitals may effectively become off limits to ordinary location-based advertising. For example, a gym engaging in advertising content as generic as “New Year fitness deals!” could be impacted simply because of its proximity to an entity that provides in-person health care services.

The bill’s broad definition could impact benign and everyday advertising activity that consumers benefit from and expect. For example, a corner store and its advertising partners may collect information about a customer’s purchases to offer the customer discounts for products they may be interested in. The corner store may learn that the customer purchased a dandruff shampoo and want to offer the customer coupons for future purchases of similar items. However, under B26-0525’s definition, a purchase of dandruff shampoo could be

⁴ B26-0525 at Sec. 2, § 28-3881(7).

⁵ See Conn. Gen. Stat. § 42-515(9); Md. Code Ann., Com. Law § 14-4601(I); Nev. Rev. Stat. § 603A.430.

⁶ B26-0525 at Sec. 2, § 28-3882(a).

interpreted to “identify” a health status, even though the corner store and its partners are not using it in that way. Washington State’s My Health My Data Act uses a similar definition of health data. After its passage, the Washington Attorney General had to issue a clarification stating that the law does not extend to ordinary personal data, such as information about toilet paper, deodorant, or mouthwash purchases, because the statutory text itself did not provide that level of clarity.⁷ The Committee should avoid adopting similar language that would create uncertainty about the scope of data covered by its law.

Offering a fitness deal or dandruff shampoo coupon at a local grocery store should not be treated the same as advertising that targets individuals based on specific, sensitive health diagnoses. The bill risks turning entire neighborhoods into commerce “dead zones,” not because advertisements are based on sensitive health data, but because the geofencing restriction applies in a manner that would chill economic activity. We encourage the Committee to carefully consider B26-0525’s potential impact on location-based advertising that consumers expect and value.

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We respectfully ask the Committee not to advance B26-0525 as proposed and would welcome the opportunity to engage further and work with you to craft amendments that would benefit District of Columbia businesses and consumers alike.

Thank you in advance for your consideration of this letter.

Sincerely,

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⁷ See FAQs on Protecting Washingtonians’ Personal Health Data and Privacy, located [here](#).